



Standard Operating Procedure: Tiered Institutional Pricing & Community Sight Access (Southern California)

SOP ID: ENR-CLN020

Version: 1.0

Effective Date: April 10, 2026

Review Date: April 10, 2026

Scope: Clinical Operations & Administrative Staff

Purpose:

The purpose of this directive is to formalize the financial framework for mobile clinical services within the Southern California market. This SOP ensures that pricing is applied with clinical integrity, administrative transparency, and alignment with Enrich’s mission of sight preservation.

Policy Statement

Enrich mandates a structured fee schedule to balance institutional partnerships with community-driven healthcare.

Note: Final invoice amounts are subject to a **Geographic Adjustment Factor (GAF)** based on the patient’s zip code to account for extended travel time and logistics in high-traffic or remote areas (e.g., High Desert or South San Diego County).

Procedure: Tier Classification

- **Premium Access Processing (\$450.00)**
 - 1. **Verification:** Upon receipt of a requisition, the Intake Coordinator shall cross-reference the referring physician/facility against the Registered Provider Database (Trello Board).
 - 2. **Eligibility:** Must be an institutional partner (e.g., USC, UCI) or a clinic with a **signed Provider Service Agreement (PSA)** on file.
 - 3. **The "Upgrade" Provision:** If a provider is unregistered, the Sales Team may offer immediate enrollment.
 - ✓ Upon execution of the PSA, the patient’s rate is retroactively adjusted to \$450.00 for the current service.
- **Community Sight Day Processing- Tier 2 (\$320.00)**
 - 1. **Trigger:** The referring physician must explicitly write "**Community Sight Day**" on the clinical requisition form.



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2. **Financial Hardship:** This rate is applied without further income verification, relying on the clinical discretion of the referring doctor.

3. **Logistics (Batching):** * These appointments are not "on-demand."

- Operations will schedule these during "Sight Day" windows (e.g., Every 2nd Tuesday for Inland Empire) to maintain the subsidized cost structure.

➤ **Standard Out-of-Network (OON) Processing - Tier 3 (\$640.00)**

- **Application:** Applied to all orders where the provider is not in the network and no "Community Sight" designation is present.
- **Direct Requests:** Any patient-initiated request without a valid physician referral automatically defaults to the Standard Rate.

Operational Workflow (Step-by-Step)

1. Requisition Intake: The Intake Coordinator reviews the incoming referral for the **Provider ID**.

2. Tier Verification:

- If Provider ID is Active → Apply **Tier I**.
- If "Community Sight" is Notated → Apply **Tier II** and assign to the next regional batch window.
- If neither → Default to **Tier III**.

3. Logistical Assessment:

The Routing Coordinator calculates the distance-based adjustment for Southern California traffic corridors (e.g., Inland Empire, High Desert).

4. Final Authorization:

A "DocStamp" is applied to the digital file, certifying the price tier before the clinical team is dispatched.

Service Inclusions & Quality Control

- **Logistics:** All tiers include RN-led intake and cold-chain logistics.
- **Documentation:** All pricing tier assignments must be logged in the CRM with a timestamp and a copy of the triggering requisition form for ISO audit purposes.
- **Review:** Regional fee schedules are reviewed annually by the CFO and Clinical Director to ensure alignment with Southern California overhead costs.



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Quality Risk Management & Compliance

- **ISO Audit Trail:** All Tier II (\$320) assignments must be backed by a physician-signed requisition. Invoices without this documentation will be flagged during internal audits.
- **Non-Discrimination:** Pricing tiers are based strictly on institutional affiliation and clinical notation, ensuring compliance with fair billing practices.
- **Cold-Chain Integrity:** Regardless of pricing tier, all Southern California specimens must adhere to the **Enrich Standard** for hand-delivered, RN-led cold-chain logistics.

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